
The Lyfe Place Aesthetics

Financial model, start-up capital and feasibility.

Medbury Healthcare · KP Plastics · Consult for Africa

A companion to the partnership pre-read

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01 / HOW TO READ THIS

A benchmark model, costed bottom-up.

This is the companion to the pre-read. It sizes the start-up capital, projects five years, and tests whether the partnership stands up. The costs are forecast bottom-up, line by line, with a 20% contingency over the build. The figures are premium-Lagos benchmarks, meant to be tuned against the scope and the site we agree.

This is a surgery-led business. Dr Kpaduwa is a plastic surgeon, so cosmetic and plastic surgery is the anchor line, with injectables, skin and dermatology compounding around it. All figures are in NGN and nominal.

The base model rents the theatre and recovery from an existing partner hospital rather than building them, which is the lightest on capital and the strongest on post-op care. That takes start-up capital to about NGN 260M. The next page details the arrangement and compares it with a standalone theatre of our own.

02 / THE THEATRE MODEL

Rent the theatre, do not build it yet.

A surgery-led model needs an operating theatre and a safe place to recover. The base case gets both from an existing hospital under a use-of-facilities agreement, rather than building and staffing a theatre from day one. The Lyfe Place holds only the consulting rooms and a procedure room, for consultations, injectables, skin, dermatology and minor procedures under local anaesthetic. Major surgery under general anaesthetic is never done there; it goes to the partner hospital's theatre.

What the partner hospital provides

- A licensed operating theatre and booked theatre time.
- Anaesthesia and theatre nursing support for each list.
- Post-operative recovery, and overnight or short-stay beds.
- An ICU and emergency backstop, so complex recoveries are safe.

How it works

- Dr Kpaduwa and the surgical team hold operating privileges at the partner and bring their own instruments, consumables, implants and patients.
- The partner bills the JV a theatre and facility fee per case, or a block rate for committed sessions, mirroring the members-plus-pay-per-use logic: commit to sessions, pay per use.
- A benchmark fee is about NGN 700k to 1.2M per cosmetic case, covering theatre, anaesthesia, recovery and one to two nights.
- A candidate partner is an Evercare-tier private hospital in Lekki or on the Island with spare theatre capacity and good recovery and ICU.

Why this is the base: it is the lightest on capital, the fastest to open, and post-op is managed inside a full hospital rather than a converted clinic. It also turns the theatre from a fixed cost the partnership carries into a variable cost it only pays when it operates. Build or buy a theatre later, once the surgical volume proves out.

02 / THE THEATRE MODEL

The two ways to get a theatre.

The same surgery-led model, costed two ways. The Lyfe Place holds the consulting and procedure rooms in both; only the theatre moves. Capital includes the 20% contingency; margin is the Year 3 steady state.

Theatre option	Capital (NGN M)	Theatre and post-op	Yr 3 margin
Partner hospital theatre (base)	~260	Host theatre, recovery, overnight and ICU backstop	~31%
Standalone with own theatre	~600 + property	Purpose-built theatre and proper post-op; later, at volume	~29%

The reading is clear. At the volumes this business runs, renting a theatre is both lighter on capital and no worse on margin than building a standalone, and it gives better post-op care from day one. A standalone theatre only earns its keep at much higher surgical volume, and it is the right move later, not at launch. The Lyfe Place holds the consulting and procedure rooms in both cases; it never holds the theatre. The Cloister could serve as a second, Ikoyi front, but it too is not a theatre.

The lean: launch on the partner-theatre model to prove the surgical volume on the lightest capital, then revisit a standalone theatre of our own once the lists are consistently full.

03 / REVENUE ASSUMPTIONS

What the top line is built on.

The revenue spine is surgery-led: cosmetic and plastic surgery is the anchor, with injectables, body contouring, skin and laser, and medical dermatology alongside, plus consultations and skincare retail. Prices are premium-Lagos benchmarks; volumes are the Year 3 steady state, reached on a ramp from launch.

Pricing and volume, Year 3 steady state

Service line	Avg fee (NGN)	Cases / yr	Revenue (NGN M)
Cosmetic and plastic surgery	5,000,000	144	720
Injectables (toxin, fillers, threads)	300,000	1,200	360
Body contouring (non-surgical)	450,000	360	162
Skin and laser	130,000	1,200	156
Dermatology (medical)	90,000	900	81
Consultations	60,000	1,500	90
Skincare product retail	by margin	-	90
Total, Year 3			1,659

Surgery at about NGN 5.0M is a blended plastic-surgery ticket across liposuction, body and breast surgery, abdominoplasty and combination cases, done at the partner theatre. Volumes assume launch into an existing base, Dr Kpaduwa's following, the Lyfe Place patients and the Medbury corporate book, and associate surgeons extending capacity when she is abroad.

04 / START-UP CAPITAL

Costed bottom-up, with 20% contingency.

The capital call to open, on the partner-theatre base case. Because surgery runs at the host hospital, there is no theatre to build or equip; the spend is the consulting and procedure front and launch. The 20% contingency sits over the whole build, and the Lyfe Place site is Medbury's and not in this figure. A standalone theatre of our own, if we build one later, adds ground-up construction plus rent or acquisition.

Item	NGN M
Front fit-out (consulting rooms, procedure room, aesthetic treatment rooms, recovery bay)	45
Aesthetic and dermatology equipment (laser, injectables and dermatology station, RF; body device phased)	48
Minor-procedure and clinical equipment (surgical sets, monitors, autoclave, emergency)	25
Power and infrastructure (over the existing site)	15
Pre-opening (recruitment, licensing, branding, launch)	30
Opening inventory (aesthetic product and consumables; implants bought per case)	18
Build subtotal	181
Contingency (20% of build)	36
Build including contingency	217
Working-capital buffer (about 2 months, cash-pay)	45
Total start-up capital, partner-theatre base	262

The aesthetic platform is one strong multi-application laser plus the injectables and dermatology station and RF microneedling, with the body-contouring device phased into Year 2 from cashflow. The minor-procedure room handles injectables, skin and local-anaesthetic work at the front; anything needing general anaesthetic goes to the partner theatre. Every line is a benchmark to firm up on the chosen site.

05 / OPERATING COST

The full annual cost load, honestly.

Year 3 steady state, built bottom-up. The theatre now shows up here, as a per-case hire rather than a fixed cost, which is the whole point of the asset-light model.

Line	NGN M	What it covers
Cost of care	514	31% of revenue: surgical consumables, implants, surgeon fees, aesthetic product
Theatre hire and host-hospital fees	130	Theatre, anaesthesia, recovery, overnight beds and ICU backstop at the partner, per case
Salaries and clinical team	100	Clinical lead, aesthetic nurses and injectors, a procedure nurse, therapists, coordinators, manager
Marketing	149	9% of revenue, digital-led premium creative
Occupancy	55	Front facility charge to Medbury, power, water, security, cleaning
Insurance, maintenance, service	40	Surgical indemnity, facility and equipment cover, laser and clinic service contracts
Admin, software, clinical waste	24	HospitlOS, office, biohazard waste disposal
C4A management fee	133	8% of revenue; C4A IP licensed in, never contributed as equity
Total operating cost	1,145	
EBITDA	514	About 31% of Year 3 revenue

Surgeon economics are a term to settle: Dr Kpaduwa's return comes largely through her 34% equity, with a capped clinical fee inside cost of care; associate surgeons are paid per list. Because the host provides theatre nursing and recovery, the JV carries no in-house theatre team, which is why salaries are lighter than an owned-theatre model.

06 / THE FIVE-YEAR MODEL

Revenue build to steady-state margin.

Surgery is the anchor from launch, run in blocks around Dr Kpaduwa's diary at the partner theatre and extended by associate surgeons. Margin ramps from the mid-twenties in the build year to the low thirties at steady state, carrying the per-case theatre hire in full.

NGN M	Year 1	Year 2	Year 3	Year 4	Year 5
Revenue	872	1,372	1,659	1,913	2,101
Cost of care (about 31%)	(270)	(425)	(514)	(593)	(651)
Theatre hire and host fees	(68)	(108)	(130)	(150)	(165)
Gross profit	534	839	1,015	1,170	1,285
Salaries and clinical team	(78)	(92)	(100)	(108)	(116)
Marketing (about 9%)	(78)	(123)	(149)	(172)	(189)
Occupancy (front, power)	(44)	(52)	(55)	(60)	(63)
Insurance, maintenance, service	(30)	(36)	(40)	(44)	(47)
Admin, software, waste	(18)	(22)	(24)	(26)	(28)
C4A management fee (8%)	(70)	(110)	(133)	(153)	(168)
EBITDA	216	404	514	607	674
EBITDA margin	25%	29%	31%	32%	32%

Cumulative EBITDA over the five years is about NGN 2,415M. Year 1 is back-half weighted; the annual figure already carries the launch ramp. Building a standalone theatre later would lift the margin at high volume, at the cost of the capital and a fixed team.

07 / FEASIBILITY

It pays back inside two years.

Test	Result	Why
Break-even revenue	about NGN 41M / month	Below the Year 1 run-rate of about NGN 73M; comfortable headroom.
Payback on start-up capital	inside Year 2	Year 1 EBITDA of about NGN 216M covers most of the roughly NGN 260M capital; cumulative clears it near month 15.
Steady-state EBITDA margin	about 31 to 32%	Full cost load including per-case theatre hire, carried honestly.
Receivables drag	minimal	Cash and card at point of care; no insurance float to finance.

The feasibility rests on things already true or cheap to secure: a credentialed plastic surgeon as the clinical anchor, an existing patient base to fill the diary, a site the group already holds, and a theatre rented rather than built. The swing factors are surgical throughput, which depends on Dr Kpaduwa's availability and theatre access, and securing a reliable partner hospital on a fair rate.

08 / VALUATION AND SCENARIOS

What the stake is worth.

On a discounted-cash-flow basis, 28% discount rate and 12% terminal growth, NGN nominal, the Base enterprise value is about NGN 1.72Bn on the partner-theatre case. The scenarios flex surgical volume, the ramp and the steady-state margin.

Scenario analysis

Scenario	What changes	EV (NGN Bn)
Bear	Slower ramp, fewer surgeries, thin associate cover	~1.1
Base	Surgery-led from launch, partner theatre, aesthetics and dermatology alongside	~1.72
Bull	Surgery scales with associate surgeons and medical-tourism inflow	~2.7

Value by stake, Base case

Party	Stake	Value (NGN M)
Medbury Healthcare Group	51%	~877
KP Plastics (Dr Chinwe Kpaduwa)	34%	~585
Consult for Africa	15%	~258
Enterprise value, Base	100%	~1,720

Sensitivity of EV (NGN Bn) to Year 5 revenue and steady-state margin

EBITDA margin	Rev 1,800	Rev 2,100	Rev 2,400
28%	1.30	1.50	1.70
32%	1.45	1.70	1.95
36%	1.65	1.90	2.20

Against a start-up capital of about NGN 260M on the base case, a Base enterprise value near NGN 1.72Bn is a strong return on the capital at risk. C4A's 15%, earned largely through structuring and the licensed platform rather than cash, is worth about NGN 258M at Base.

09 / WHAT MOVES THE NUMBERS

Risks, and the decisions that set them.

The theatre partner is the first thing to lock. The base model rents theatre and recovery from a host hospital, so securing a reliable partner with spare capacity and good recovery and ICU, on a fair per-case rate, is the decisive early move. Mitigated by choosing an Evercare-tier partner and negotiating block-sessional access; owning a theatre later removes the dependence.

Surgical throughput is the operating swing factor. Surgery is the largest revenue line and the most sensitive to Dr Kpaduwa's availability and theatre access. Mitigated by associate surgeons, a resident clinical lead, and booking surgery in blocks when she is in-country.

Costs are forecast in full, not flattered. The per-case theatre hire is a real line, sized in full, and the build carries a 20% contingency. The steady margin near 31% reflects that honesty; it is not a number to be talked upward without cutting real cost.

Consumables and implants carry FX exposure. Surgical consumables, implants and devices are imported. Priced in NGN with pass-through, and stock bought ahead of naira moves; the cost-of-care ratio is the line to watch.

Next steps

1. Secure the theatre partner: agree the host hospital, the per-case rate and the recovery and overnight terms. This anchors the model.
2. C4A firms every capital line on the front fit-out, to a committed budget with the contingency held.
3. Term sheet, then shareholder agreement; incorporate, fit out, recruit, soft-launch.

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Figures are benchmark estimates for discussion, grounded in Lagos premium cosmetic-surgery and aesthetics economics, forecast bottom-up with a 20% build contingency, and to be firmed against the chosen site and theatre partner. Not a forecast, an audit, or a binding offer. Shared in confidence with Dr Itunu Akinware, Dr Chinwe Kpaduwa and Medbury leadership.